

Vaginal Bleeding During Late Pregnancy

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During late pregnancy (after 20 weeks), 3 to 4% of women have vaginal bleeding. Bleeding may be a signal that there is a risk of losing the baby or of bleeding excessively (hemorrhaging). In rare cases, so much blood is lost that blood pressure becomes dangerously low (causing **shock**) or small blood clots form throughout the bloodstream (called **disseminated intravascular coagulation**).

Women with vaginal bleeding during late pregnancy should go to the doctor to be evaluated promptly, because the bleeding may be associated with complications that threaten the safety of the woman and/or the fetus.

Causes of Vaginal Bleeding in Late Pregnancy

The **most common cause** of bleeding during late pregnancy is

- The start of labor

In late pregnancy, some women pass the mucus plug (collection of mucus in the cervix) through the vagina. The mucus plug is usually a sticky small ball and often has streaks of blood in it. Passing the mucus plug is usually a signal that labor will begin within the next week. Usually, labor starts with a small discharge of blood mixed with mucus from the vagina. This discharge, called the bloody show, occurs when small veins are torn as the cervix begins to open (dilate), enabling the fetus to pass through the vagina. The amount of blood in the discharge is small.

More serious but less common causes (see table

Some Causes and Features of Vaginal Bleeding During Late Pregnancy)

- **Placenta previa**

- [Vasa previa](#) (rare)
- [Placental abruption](#)
- [Rupture of the uterus](#) (rare)

In **placenta previa**, the placenta is attached to the lower rather than the upper part of the uterus. When the placenta is lower in the uterus, it may partly or completely block the cervix (the lower part of the uterus), which the fetus must pass through. Bleeding may occur without warning, or it may be triggered when a practitioner examines the cervix to determine whether it is dilating or whether labor has started. Placenta previa accounts for approximately 20% of bleeding during late pregnancy and is most common during the 3rd trimester. The placenta may be low-lying earlier in pregnancy, but the placenta usually moves out of the way on its own before the 3rd trimester.

Vasa previa is a rare condition in which the blood vessels that provide blood to the fetus (through the umbilical cord) grow across the cervix, blocking the fetus's passageway. When labor starts, these small blood vessels may be torn, depriving the fetus of blood. Because the fetus has a relatively small amount of blood, loss of even a small amount can be serious, or even fatal, for the fetus.

In **placental abruption**, the placenta detaches from the uterus before delivery. What causes this detachment is unclear, but it may occur because blood flow to the placenta is inadequate. Sometimes the placenta detaches after an injury, as may occur in a car crash. Bleeding may be more severe than it appears because some or most of the blood may be trapped behind the placenta and thus not be visible. Placental abruption is the most common life-threatening cause of bleeding during late pregnancy, accounting for approximately 30% of cases. Placental abruption may occur at any time but is most common during the 3rd trimester.

Rupture of the uterus may occur during labor. It almost always occurs in women whose uterus has been damaged and contains scar tissue. Such damage may occur during a cesarean delivery or other uterine surgery (such as myomectomy to remove [uterine fibroids](#)).

Bleeding may also result from disorders unrelated to pregnancy.

Risk factors

Various conditions (risk factors) increase the risk of disorders that can cause bleeding during late pregnancy.

For **placenta previa**, risk factors include the following:

- A [cesarean delivery](#) in a previous pregnancy

- One or more previous pregnancies
- A [pregnancy with more than one fetus](#)
- [Placenta previa](#) in a previous pregnancy
- Age over 35
- Cigarette smoking
- [In vitro fertilization](#) (fertilization of the egg in a laboratory and placement of the embryo in the uterus)

For **vasa previa**, risk factors include the following:

- A placenta located low in the uterus
- A placenta that is divided into sections
- A [pregnancy with more than one fetus](#)
- [In vitro fertilization](#)

For **placental abruption**, risk factors include

- [High blood pressure](#)
- Age over 35
- One or more previous pregnancies
- [Cigarette smoking](#)
- Use of [cocaine](#)
- [Placental abruption](#) in a previous pregnancy
- A recent abdominal injury (as may occur in a car accident)

For **rupture of the uterus**, risk factors include the following:

- A [cesarean delivery](#) in a previous pregnancy
- Any prior surgery involving the uterus
- Age over 30
- Previous infections of the uterus
- Artificial starting (induction) of labor
- Injury, as may occur in a car crash
- Birth of more than five babies
- Pregnancies that are too close together
- [Placenta accreta](#) (a placenta that grows too deeply into or through the uterus)

Evaluation of Vaginal Bleeding in Late Pregnancy

Doctors focus on ruling out potentially serious causes of bleeding (such as [placental abruption](#), [placenta previa](#), [vasa previa](#), and [rupture of the uterus](#)). If the evaluation rules out these more serious causes, doctors usually diagnose the most common cause—the start of labor, indicated by the bloody show.

Warning signs

Any vaginal bleeding late during pregnancy is considered a warning sign, except for the mucus plug or bloody show, which only has a small amount of blood and does not last long.

Doctors are particularly concerned about women who have fainted, feel light-headed, or have a rapid heart rate—symptoms that suggest very low blood pressure.

The following symptoms are also cause for concern:

- A tense, tender uterus
- No heartbeat or a slow heart rate in the fetus
- Labor that stops and loss of muscle tone in the uterus

When to see a doctor

A woman with vaginal bleeding during late pregnancy should go to the hospital immediately. However, if she suspects that the bleeding is the bloody show, she should call the doctor first. The doctor can determine how quickly she needs to be seen based on the amount and duration of bleeding and the presence of signs of labor.

What the doctor does

Doctors first ask questions about the bleeding and other symptoms and about the medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the pain and the tests that may need to be done (see table [Some Causes and Features of Vaginal Bleeding During Late Pregnancy](#))

Doctors ask about the bleeding:

- How long it lasts
- How severe it is

- What color the blood is
- Whether the woman has or has had other symptoms (such as abdominal pain, light-headedness, or fainting)

The woman is asked about her pregnancies: how many times she has been pregnant, how many children she has had, and whether she has had any miscarriages or abortions or any problems in previous pregnancies. The woman is asked whether the membranes have ruptured (whether her water broke), usually a sign that labor is starting or has started.

Doctors ask about conditions that increase the risk of the most common and serious causes of bleeding and about [risk factors for these causes](#), particularly a cesarean delivery in a previous pregnancy.

During the physical examination, doctors first check for signs of substantial blood loss, such as a rapid heart rate and low blood pressure. They also check the heart rate of the fetus and, if possible, start monitoring the fetus's heart rate constantly (with [electronic fetal heart monitoring](#)). Doctors gently press on the abdomen to determine how large the uterus is, whether it is tender, and whether its muscle tone is normal. They then do a pelvic examination. They examine the cervix using an instrument that spreads the walls of the vagina apart (speculum).

Normally when delivery is near, doctors examine the cervix with a gloved hand (digital cervical examination) to determine how dilated the cervix is and how the fetus is positioned (see [Labor](#)). However, if bleeding occurs during late pregnancy, ultrasonography is done to check for placenta previa and vasa previa before this examination is done. If either disorder is present, the examination is not done because it may make the bleeding worse. Women who know they have placenta previa or vasa previa should feel empowered to remind clinicians of their diagnosis and refuse having a digital cervical examination. (Careful speculum examination can be done.)



Some Causes and Features of Vaginal Bleeding During Late Pregnancy

Cause	Common Features*	Diagnostic Approach†,‡
<u>Labor</u>	Begins with passage of a small amount of blood, sometimes mixed with mucus (mucus plug or bloody show) Uterine contractions at regular intervals plus opening (dilation) and thinning and pulling back (effacement) of the cervix	Pelvic examination and monitoring of woman's vital signs Fetal heart rate monitoring
<u>Placental abruption</u> (premature detachment of the placenta from the uterus)	Pain or tenderness when the uterus is touched Passage of dark, clotted, or bright red blood that can be light or heavy bleeding Sometimes low blood pressure in the woman, with fainting, light-headedness, or a rapid heart rate	Pelvic examination and monitoring of woman's vital signs Fetal heart rate monitoring Complete blood cell count and blood tests to determine whether blood is clotting
<u>Placenta previa</u> (an abnormally located placenta)	Sometimes an abnormal heart rate in the fetus	Often ultrasonography Complete blood count Ultrasonography